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6-8-2025

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Recommended Citation

Mitchell, Meagan and Luk, Jenifern, "Surgeon General's Advisory on Social Media Use and Youth Mental Health: A Public Health Crisis" (2025). *URGENT Matters*. Paper 56.
https://hsrc.himmelfarb.gwu.edu/smhs_URGENT_Matters/56

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Surgeon General's Advisory on Social Media Use and Youth Mental Health: A Public Health Crisis

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June 8, 2025

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On June 14, 2024, U.S. Surgeon General Dr. Vivek Murthy published an [op-ed](#) in the New York Times calling for warning labels on social media platforms, highlighting the growing evidence of their negative effects on youth mental health. Dr. Murthy emphasized that these harms are not the result of inadequate parenting, but stem from the widespread adoption of powerful technologies without sufficient safety, transparency, or accountability.

Social media use is nearly universal among teenagers, with up to 95% of youth ages 13–17 reporting use and more than a third using it almost constantly. While some benefits exist—such as community building for marginalized youth and opportunities for identity exploration—social media also presents significant risks. Its [impact on mental health](#) depends on the amount of time spent, the nature of the content consumed, and individual vulnerabilities. Notably, adolescents who spend more than three hours per day on social media have [double the risk](#) of experiencing depression and anxiety, while limiting use to less than thirty minutes daily can reduce depressive symptoms. [Girls](#) may be particularly [vulnerable](#) to these negative [effects](#).

Harmful impacts of social media include expo-

sure to inappropriate or dangerous content, increased body dissatisfaction, cyberbullying, and predatory behaviors. Platform features designed to maximize user engagement—such as autoplay, infinite scroll, and push notifications—can foster compulsive use. This compulsive behavior is associated with sleep disruption, attention difficulties, and, in some cases, changes in brain structure similar to those seen in gambling addiction. Excessive use may also contribute to academic and social decline, with some studies suggesting a link to increased [ADHD](#) risk.

Despite these concerns, important [knowledge gaps](#) remain. Research is needed to better understand how digital interactions compare to in-person connections, which types of content are most harmful or beneficial, and what protective factors might mitigate risks. It is also unclear how social media's impact varies across developmental stages and what interventions are most effective in promoting healthy use.

Healthcare professionals play a critical role in addressing this issue. Routine screening for problematic social media use should be incorporated into adolescent visits, watching for signs such as inability to limit use, interference with daily responsibilities, or [negative](#) impacts on relationships and

academic performance. Providers should counsel families on limiting social media—particularly before bedtime to preserve healthy sleep—and encourage digital literacy for both adolescents and parents. Engaging communities to set shared norms and referring families to resources like the Log Off movement and Wired Human can further support healthy technology use.

In conclusion, protecting youth mental health in the digital age requires coordinated action from healthcare providers, policymakers, and communities. By integrating digital health screening into routine care, educating families, and advocating for safer, evidence-based policies, the medical community can play a vital role in mitigating the risks of social media and supporting the well-being of children and adolescents.

The authors have no conflicts of interest to disclose.

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